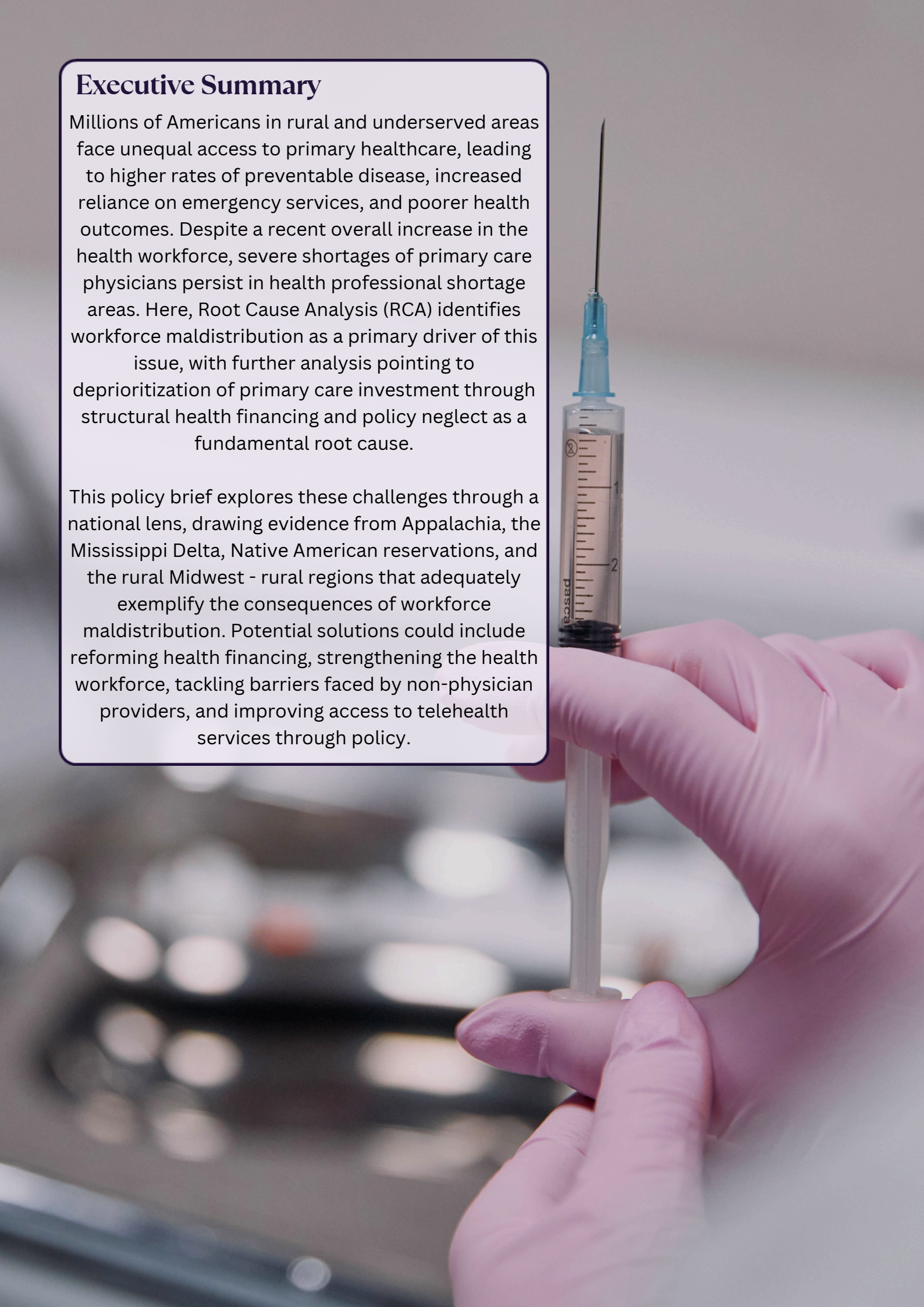
Bridging the Gap: Addresing Primary Care Access in the United States

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Executive Summary

Millions of Americans in rural and underserved areas face unequal access to primary healthcare, leading to higher rates of preventable disease, increased reliance on emergency services, and poorer health outcomes. Despite a recent overall increase in the health workforce, severe shortages of primary care physicians persist in health professional shortage areas. Here, Root Cause Analysis (RCA) identifies workforce maldistribution as a primary driver of this issue, with further analysis pointing to deprioritization of primary care investment through structural health financing and policy neglect as a fundamental root cause.

This policy brief explores these challenges through a national lens, drawing evidence from Appalachia, the Mississippi Delta, Native American reservations, and the rural Midwest - rural regions that adequately exemplify the consequences of workforce maldistribution. Potential solutions could include reforming health financing, strengthening the health workforce, tackling barriers faced by non-physician providers, and improving access to telehealth services through policy.

1.0 - Introduction

The US' healthcare system (Appendix 1) is renowned for its advanced medical services. However, millions of Americans struggle to access primary care (PC) due to the uneven distribution of healthcare professionals (1). This issue is more severe in rural and medically underserved urban areas, where a shortage of primary care physicians (PCPs) results in reduced access to preventative care, higher rates of unmanaged chronic disease, increased emergency department (ED) utilization, and worse overall health outcomes (Figure 1) (2–6).

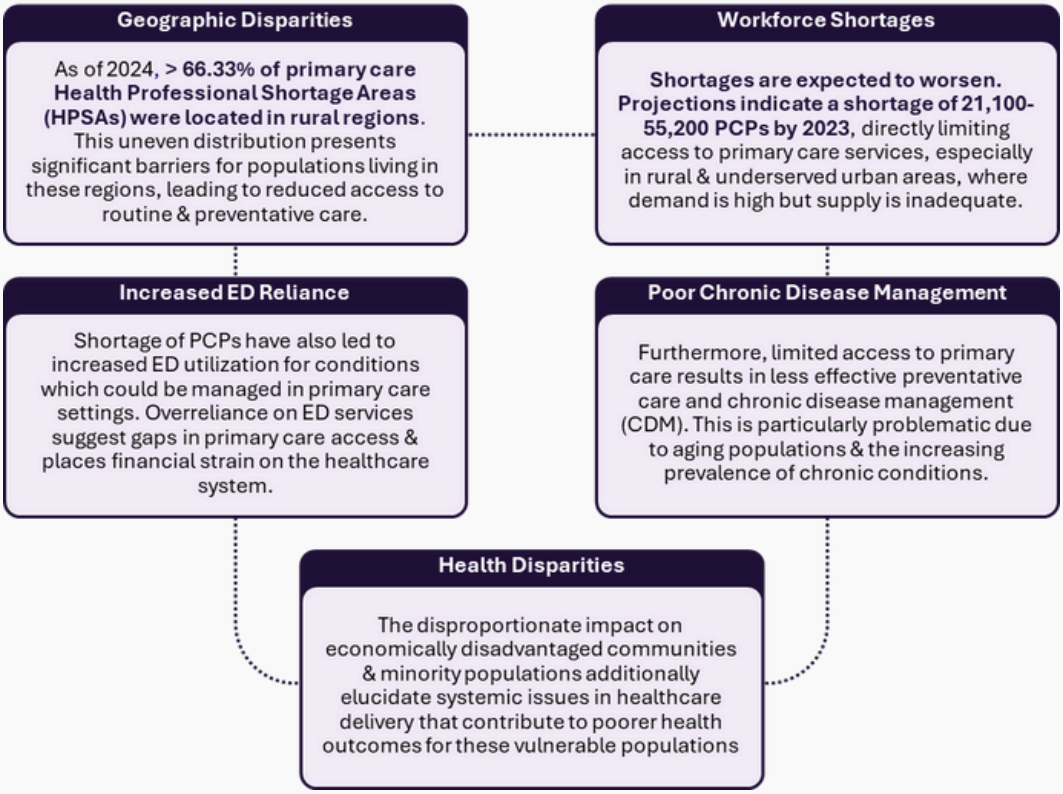


Figure 1 - Markers of Limited Access to Primary Care (2-6).

Over 100 million Americans reside in designated Primary Care Health Professional Shortage Areas (HPSAs), the majority of which are situated in rural regions (1). Compared to urban residents, rural populations experience a 40% higher rate of preventable hospital admissions and face a greater burden of poorly managed chronic conditions such as diabetes and hypertension (2,7). With limited access to PC providers, these populations are three times more likely to rely on EDs for non-emergency conditions, placing financial strain on the nation's healthcare system (7). While rural life expectancy is 2-3 years lower than in more urban areas, multiple factors beyond healthcare access contribute to this gap, including the wider determinants of health (7). Nevertheless, limited access to primary and preventative care remains a significant challenge to achieving Universal Health Coverage (UHC).

America's incapacity to provide adequate healthcare for its population elucidates the presence of a major bottleneck in its health workforce. This policy brief therefore aims to identify the key causal pathways associated with suboptimal health workforce performance that leads to poor PC access across the nation. Drawing evidence from multiple rural regions with high PC shortages, it seeks to offer insights into systemic barriers and potential solutions at the national level (Figure 2).

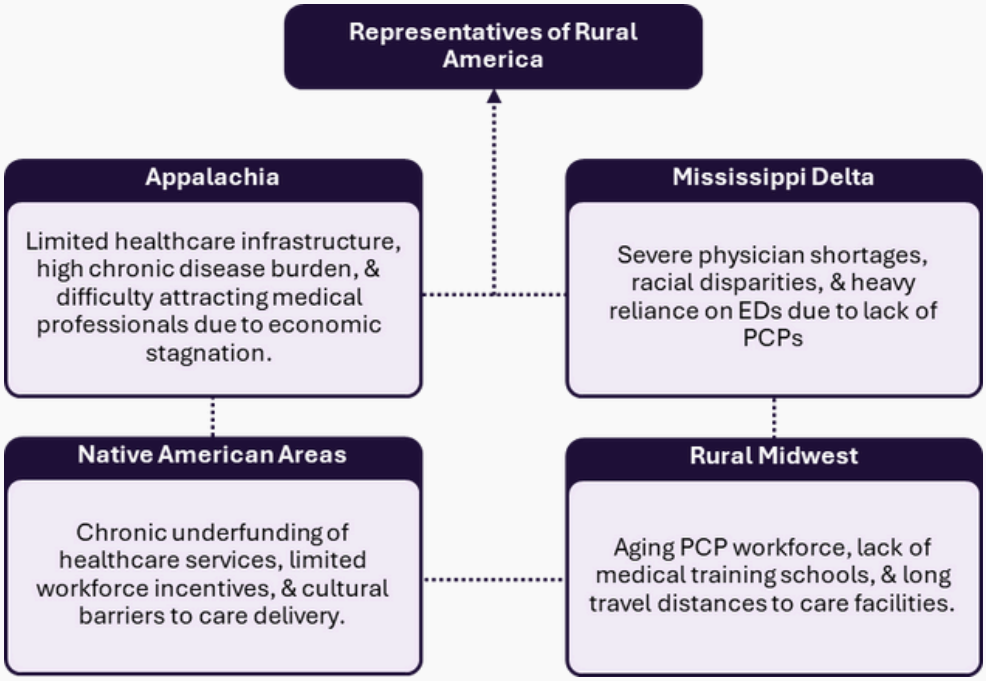


Figure 2 - Regions that serve as effective representations of rural America (2).

2.0 - Methodology

A literature review was conducted to elucidate the many reasons why Americans struggle to access PC. Then, a Fishbone Analysis was employed to identify contributing underlying causes from the perspectives of all key stakeholders. Based on the most common emerging themes, the Five Whys framework was then used to further isolate the most significant root cause contributing to the bottleneck in the US' health workforce.

3.0 - Fishbone Analysis Fndings

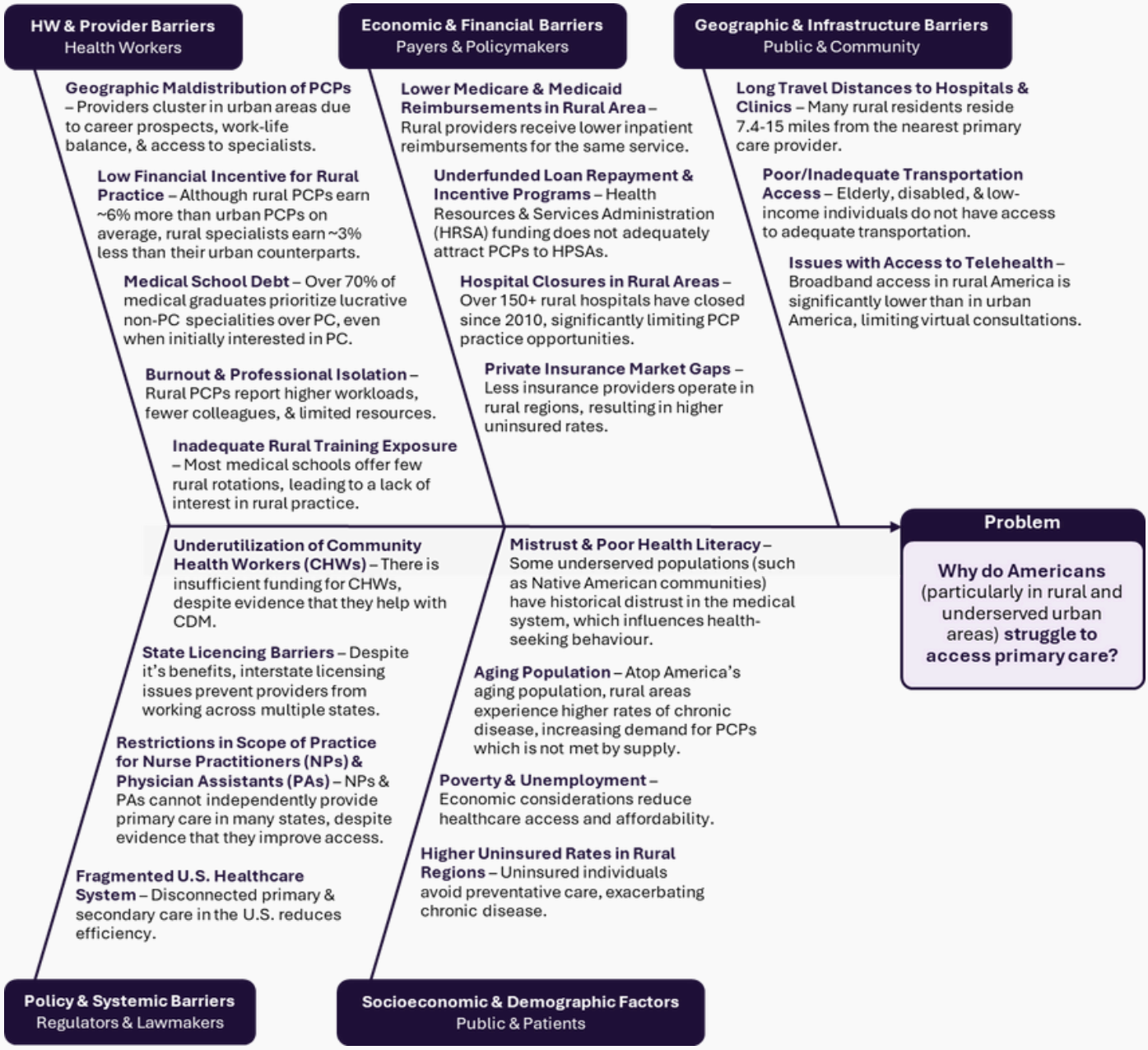


Figure 3 - A Fishbone Analysis of the contributing factors to why Americans struggle to access primary care (2,8-35).

The Fishbone Analysis identified several contributing causes to America's inadequate access to PC. Some causes were found to be interdependent, while others were not.

3.1 - Maldistribution of Primary Care Providers

A maldistribution of PCPs was found to be a key driver of the disparities outlined in Figure 1. While urban areas have between 84-110 PCPs per 100,000 of the population, rural regions have only 68 per 100,000, a gap that has widened over the past decade (8,9). One reason for this are financial disincentives, which discourage physicians from pursuing careers in PC. Despite a 6% higher average salary for rural PCPs, specialists earn significantly more, influencing medical graduates to prioritize non-PC fields (10). Moreover, medical school debt discourages long-term commitments to lower-paying positions (11,12). Those who do enter PC in underserved areas face professional isolation, heavier workloads, and subsequent burnout, further exacerbating health workforce shortages (13–15).

3.2 - Economic & Financial Barriers

Economic barriers are another contributor to reinforced provider shortages. Lower inpatient reimbursement rates in rural areas disincentivize PC practice, with rural hospitals receiving 90.6% that of Traditional Medicare rates compared to their urban counterparts (18). Furthermore, the closures of 150+ rural hospitals since 2010 have significantly attenuated PC access and practice opportunities (22). Meanwhile, low private insurance market competition has led to higher uninsured rates in rural communities (12.6%) compared to more urban ones (10.9%), discouraging timely care utilization (23).

3.3 - Geographic, Socioeconomic & Demographic Factors

Geographic and infrastructure barriers compound these issues. A national survey found rural residents often live 7.4-15 miles from their nearest PC provider, creating logistical hurdles to care, particularly for disabled, low-income, and elderly populations with inadequate transportation options (2,24,25). This is of particular concern given the US' ageing population, which particularly in rural areas, is increasing unmet demand (6). While telehealth has the potential to bridge these gaps, low broadband access, poor health literacy, and historical distrust among Native American and other marginalized communities remain of significant challenge in rural America, reducing health and preventative care engagement and contributing to delayed treatment-seeking behaviours (26,36).

3.4 - Structural & Policy Failures

Beyond workforce and infrastructure challenges, systemic policy failures contribute to fragmented and inefficient service delivery. The US healthcare system prioritizes specialist-driven care, offering higher reimbursement rates for procedures over PC services (27,37). In addition, state-level restrictions on nurse practitioners (NPs) and physician assistants (PAs) limit their ability to provide independent PC, despite evidence that they improve access (38). Interstate licencing barriers further restrict provider mobility, exacerbating regional disparities. Meanwhile, community health workers (CHWs), who play a fundamental role in CDM and preventative care, remain underfunded despite their proven effectiveness (29).

4.0 - Five Whys Findings

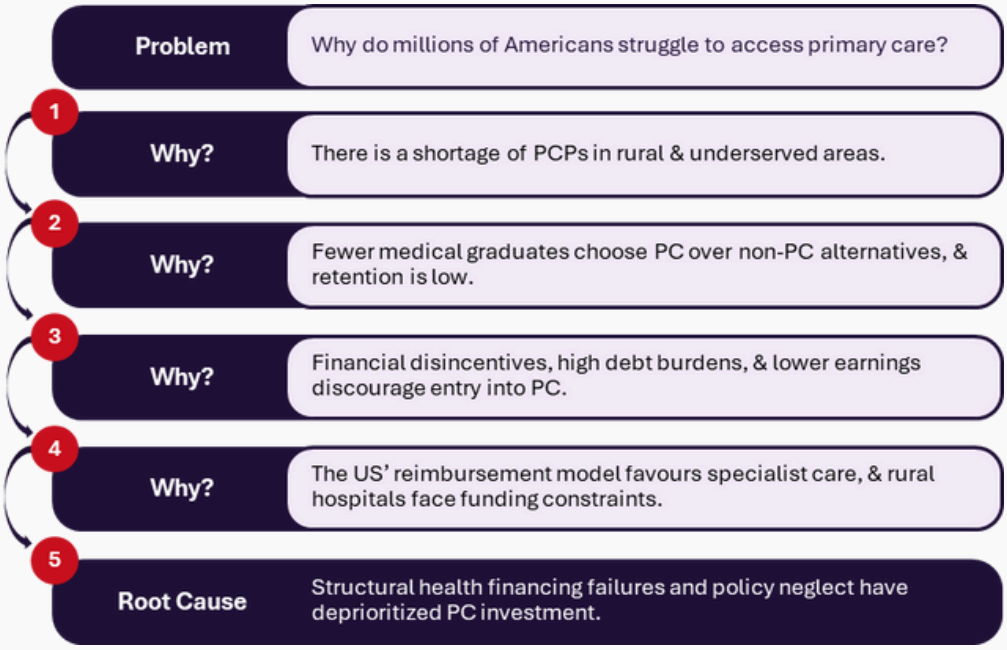


Figure 4 - A Five Whys Analysis of the contributing factors to why Americans struggle to access primary care.

Root cause analysis (RCA) of these issues through the Five Whys framework highlights financial and systemic underinvestment in PC as the fundamental driver of the identified disparities. The US' reimbursement model structurally favours non-PC specialties, leaving PC underfunded and unattractive to medical graduates. Without targeted health workforce reforms, funding realignment, and policy interventions, access barriers will persist, disproportionately affecting underserved communities, and further straining the healthcare system.

5.0 - Policy Recommendations

To address this structural underinvestment and improve healthcare access for underserved populations, the following policy recommendations outlined in Table 1 suggest precise, feasible, and relevant steps policymakers could take. Implementation of these will require a coordinated effort between federal agencies, state governments, medical institutions, and community stakeholders. Policymakers should apply the relevant combination of recommendations as per each state's needs. They should establish measurable benchmarks, monitor policy impacts, and adjust strategies as needed to ensure optimal health systems development and sustainable improvements in PC accessibility across the United States.

Table 1 - Policy recommendations to address America's poor access to primary care (39-43). *See Table 2 for a more detailed breakdown of what each policy recommendation could entail.*

Policy Recommendation	Reasoning	Evidence
Reform Health Financing to Strengthen Primary Care (PC) Investment	The current reimbursement system prioritizes specialty care, disincentivizing medical graduates from entering PC. Aligning financial incentives will make PC a more attractive & sustainable career choice.	The US currently spends 5-7 cents per every healthcare dollar on PC, compared to 13 cents in other high-income countries (HIC) (39). Increased investment in PC has been linked to improved health outcomes, better equity, & a reduced healthcare burden (39).
Expand & Strengthen the PC Workforce	Addressing the provider shortage calls for increasing training capacity & ensuring more physicians enter & remain in PC roles.	Rural training programs have proven effective; 25% of physicians practice in rural areas following the completion of such programs (40).
Reduce Barriers for Non-Physician Providers to Expand Access	Restrictive policies limit the ability of nurse practitioners (NPs), physician assistants (PAs), & community health workers (CHWs) to deliver care, despite evidence that they effectively improve assess & outcomes.	Strong evidence suggests expanded use of non-physician providers (particularly for routine care) has the potential to improve PC access & reduce costs (41). Studies show no statistical difference in quality of care provided by NPs & PAs compared to PCPs (41).
Bolster Infrastructure & Improve Accessibility	Geographic & financial barriers limit PC access in rural areas. Investing in infrastructure will reduce these barriers.	Telehealth has long demonstrated transformative potential, particularly in enhancing healthcare accessibility for more rural areas (42).
Strengthen Preventative & Community Based Care Models	Investing in preventative care will reduce long-term healthcare costs & improves population health outcomes.	Preventative PC has been documented to play an integral role in health promotion & disease prevention, with high uptake reducing unplanned hospitalisations (43).

Table 2- Policy recommendations to address America's poor access to primary care. *Stakeholders have varying levels of interest and power, and their potitions are not stagnant.*

Policy	Steps	Reasoning	Stakeholders Responsible	M&E
Reform Health Financing to Strengthen PC Investment	Increase Medicare & Medicaid reimbursement rates for PCPs	Low reimbursement rates were found to contribute to provider shortages and discourage physicians from working in underserved areas	Congress; Centers for Medicare & Medicaid (CMS)	Analyse payment changes & impact on provider retention in underserved areas
	Expand value-based payment models nationwide	Moving towards value-based care encourages prevention & CDM, reducing long-term healthcare costs	CMS; Private Insurers	Evaluate shifts in patient outcomes & cost savings associated with preventative care
	Bolster loan repayment incentives for PCPs in underserved areas	Graduates face high student debts, making PC less viable compared to non-PC specialities. Better incentives will improve workforce distribution	Health Resources & Services Administration (HRSA); NHSC; State Governments	Assess program enrolment growth & long-term retention rates of PCPs in shortage areas
Expand & Strengthen the PC Workforce	Increase federally funded residency slots for PC	Residency shortages limit supply of new PCPs. Broadening training opportunities will help mitigate shortages in more underserved areas	Congress; HRSA	Measure growth in PC training slots & geo distribution of graduates
	Mandate medical schools to support PC training	Medical schools often prioritize specialized training. Tying federal funding to PC requirements will hopefully increase downstream PCPs	Department of Education; Medical Schools	Track % of graduates entering PC fields
	Expand rural residency training programs	Physicians trained in rural settings will hopefully be more likely to remain in those areas	HRSA; Association of American Medical Colleges (AAMC)	Monitor impact on physician retention in rural areas
Reduce Barriers for Non-Physician Providers to Expand Access	Unstrict practice authority for NPs & PAs	Many states still restrict NPs & Pas from practicing independently, reducing care availability in shortage areas	State Governments; Nursing Boards	Monitor service utilization rates in states with expanded scope of practice laws
	Implement a national licencing compact for telehealth providers	Although telehealth has the potential to alleviate workforce shortages, state licencing barriers limit provider reach	Congress; Federation of State Medical Boards	Monitor interstate telehealth usage & access in rural communities
	Ramp up funding for CHWs	CHWs have been proven effective in managing chronic diseases and engaging in underserved populations, yet funding remains insufficient	Human & Heath Services (HHS); State Health Departments	Track CHW program enrolment & impact on preventative care utilization
Bolster Infrastructure & Improve Accessibility	Invest in rural broadband to facilitate telehealth services	Lack of internet access limits telehealth adoption, exacerbating care disparities	Federal Communications Commission (FCC)	Track telehealth adoption rates
	Increase funding for non-emergency medical transportation (NEMT)	Transportation barriers contribute to missed appointments & delayed care, worsening health outcomes	CMS; Department of Transportation (DOT); State Medicaid Agencies	Track patient utilization of NEMT services & missed appointment rates
	Bolster federally qualified health centers (FQHCs) & rural health clinics	Such centres are essential for underserved populations, but require additional support to expand services	HHS; State Governments	Evaluate clinic expansion & patient FQHC service statistics

Policy	Steps	Reasoning	Stakeholders Responsible	M&E
Strengthen Preventative & Community Based Care Models	Establish community-based PC initiatives	Local healthcare hubs (& potentially mobile clinics) improve access for high-need populations	HHS; Local Health Departments	Assess impact of (mobile health clinics), school clinics, & local health hubs
	Host public awareness campaigns on preventative care & health literacy	Public awareness can increase engagement with preventative care services, reducing healthcare distrust, costs, & disease burden	HHS; CDC; Public Health Agencies	Measure effectiveness through changes in preventative service uptake & chronic disease prevention rates

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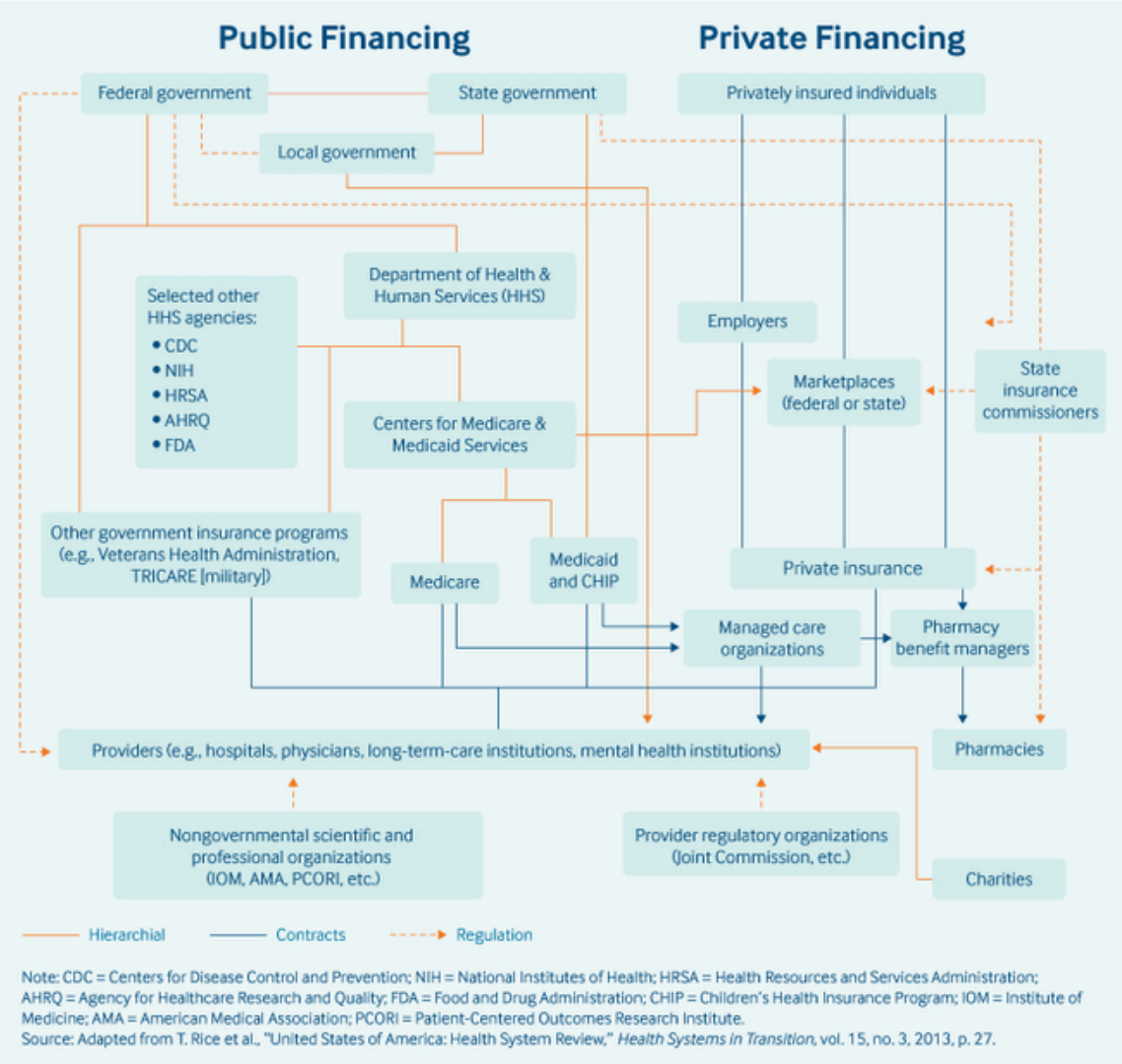
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7.0 - Appendices

Appendix 1 - Organisation of the United States' Healthcare System (44).



Note: CDC = Centers for Disease Control and Prevention; NIH = National Institutes of Health; HRSA = Health Resources and Services Administration; AHRQ = Agency for Healthcare Research and Quality; FDA = Food and Drug Administration; CHIP = Children's Health Insurance Program; IOM = Institute of Medicine; AMA = American Medical Association; PCORI = Patient-Centered Outcomes Research Institute.
Source: Adapted from T. Rice et al., "United States of America: Health System Review," *Health Systems in Transition*, vol. 15, no. 3, 2013, p. 27.

